

Crying in Children

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All infants and young children cry as a form of communication. It is the only way they have to express a need. Thus, most crying is in response to hunger, discomfort (such as that due to a wet diaper), fear, or [separation from parents](#). Such crying is normal and typically stops when the needs are met—for example, when infants are fed, burped, changed, or cuddled. This crying tends to occur less often and for shorter times after children are 3 months old.

Excessive crying refers to crying that continues after caregivers have attempted to meet routine needs or crying that continues for longer than usual for a given child.

Causes of Crying in Children

More than 95% of the time, there is no specific medical disorder that causes excessive crying. Although such crying is stressful for parents, children eventually settle down and stop crying on their own.

Fatigue is a common cause of crying in infants.

Between 6 months and 3 years of age, crying at night is often due to difficulty falling back to sleep after normal night awakenings. Falling back to sleep on their own is especially difficult for children who are used to falling asleep under certain conditions such as while being rocked or with a pacifier.

Nighttime fears are common after age 3 years. The particular fears usually depend on the child's age and stage of emotional and physical development. Sometimes children aged 3 to 8 years cry fearfully in the middle of the night and do not seem to be awake or able to be comforted. They also have no memory of a dream or of the crying when they wake in the morning. These episodes of crying are called [night terrors](#).

Did You Know...

- More than 95% of the time, there is no specific medical disorder that causes excessive crying.

Medical disorders

Less than 5% of the time, excessive crying is caused by a medical disorder. Some disorders are not immediately dangerous. Such causes of crying include [gastroesophageal reflux](#), hair wrapped around a finger, toe, or penis (hair tourniquet), a scratch on the surface of the eye (corneal abrasion), an [anal fissure](#), and a [middle ear infection](#).

Less commonly, a serious disorder that requires immediate attention is the cause. Such disorders include a blocked intestine caused by [intussusception](#) (sliding of one segment of intestine into another) and [volvulus](#) (twisting of the intestine), as well as [heart failure](#), [meningitis](#), and head injuries that cause bleeding within the skull. Infants with such severe disorders often have other symptoms (such as [vomiting](#) or [fever](#)), which alert parents to the presence of a more serious problem. However, sometimes excessive crying is the first sign.

Colic is excessive crying that has no identifiable cause and that occurs at least 3 hours a day for more than 3 days a week for more than 3 weeks. Colic typically occurs in infants approximately 6 weeks to 3 or 4 months old.

Evaluation of Crying in Children

Doctors try to identify any medical disorder that may be causing an infant's persistent crying.

Warning signs

Certain symptoms are cause for concern and suggest that a medical disorder is causing the crying:

- Difficulty breathing
- Bruises or swelling over the head or other parts of the body
- Abnormal movements or twitching of any body part



- Extreme irritability (normal handling or movement causes crying or distress)
- Continuous crying, especially if it is accompanied by a fever
- A red and/or swollen scrotum
- Fever in an infant under 8 weeks old

When to see a doctor

Children should be evaluated by a doctor right away if they have any warning signs, if they are vomiting, if they have stopped eating, or if parents notice swelling of the abdomen, a red and/or swollen scrotum, or any unusual behavior (in addition to the crying).

If children without such signs appear well otherwise, parents can try typical measures such as feeding, burping, changing, and cuddling. If crying continues after such measures, parents should call the doctor. The doctor can help parents determine how quickly the child needs to be evaluated.

What the doctor does

Doctors first ask questions about the child's symptoms and medical history. Doctors then do a physical examination. What they find during the history and physical examination often suggests a cause of the crying and the tests that may need to be done (see table [Some Medical Disorders That Cause Excessive Crying in Infants with Fever or Other Young Children](#)). Infants with fever often have an infection, those with difficulty breathing may have a heart or lung disorder, and those with vomiting, diarrhea, or constipation may have a digestive disorder.

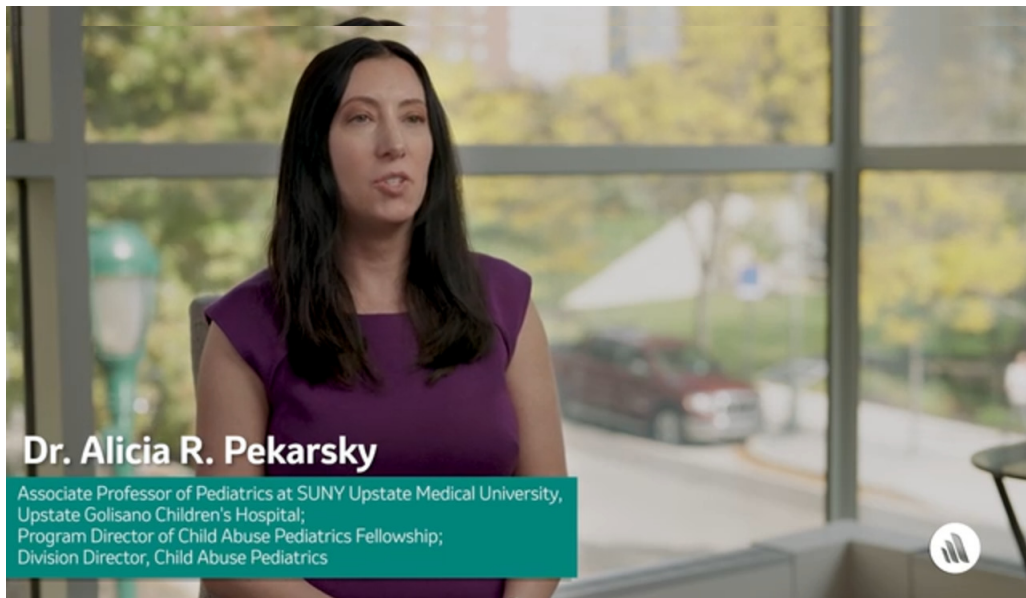
Doctors ask about the crying:

- When it started
- How long it lasts
- How often it happens
- Whether it is related to feeding or bowel movements
- How infants respond to efforts to soothe them

Parents are asked about recent events that may explain the crying (such as recent immunizations, injuries, and illnesses), and about medications given to the infant. Doctors also ask questions to learn how well the parents are bonding with the infant and managing the infant's needs.

Testing

Tests may or may not be needed depending on the infant's symptoms and the causes that doctors suspect. If the doctor's examination does not suggest a serious disorder, tests are not usually done, but doctors may schedule a follow-up visit to reevaluate the infant.



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A physical examination is done to check for symptoms of disorders that can cause discomfort or pain. Doctors look particularly at the child's eyes for a corneal abrasion and at fingers, toes, and the penis for a hair tourniquet.

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Some Medical Disorders That Cause Excessive Crying in Infants and Young Children

Cause*	Common Features†	Tests
Heart disorders		
Heart failure‡ or an abnormal heart rhythm‡	Difficulty breathing, difficulty feeding, and excessive sweating Often an abnormal heart sound detected during a doctor's examination	Chest x-ray ECG Echocardiography
Digestive disorders		
Allergy to cow's milk protein	Vomiting	Stool tests
	Diarrhea or constipation	Symptoms that lessen when the formula is changed or (for breastfed infants) cow's milk protein is eliminated from the mother's diet
	Poor feeding	Possibly endoscopy, colonoscopy, or both
	Weight loss, poor growth, or both	
	Blood in stools	
	Rash	
Constipation:	Hard, less frequent bowel movements that are difficult to pass	A doctor's examination
	Sometimes apparent pain during a bowel movement	
	Sometimes tears (fissures) in the anus	
Gastroesophageal reflux	Symptoms that occur after feeding, including fussiness or	A doctor's examination



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